

Mecobamin

Name of Product

Mecobamin Coated Tablet 500 mcg

Description of Product

Mecobamin tablet is available in white, round shape tablet and embossed with 'NOVA'. Each tablet contains 500.00 mcg of methylcobalamin.

Content

Each tablet contains:

Methylcobalamin 500 mcg

Pharmacodynamics

Methylcobalamin is the active form of vitamin B12 that acts as a cofactor for methionine synthase in the conversion of homocysteine to methionine, thus lowering blood levels of homocysteine. Methylcobalamin acts as a methyl donor and participates in the synthesis of S-adenosylmethionine, a nutrient that has powerful mood elevating properties.

Methylcobalamin donates methyl groups to the myelin sheath that insulates nerve fibers and regenerates damaged neurons. In a B12 deficiency, toxic fatty acids will destroy the myelin sheath but high enough doses of B12 can repair it.

Pharmacokinetics

Evidence indicates methylcobalamin is utilized more efficiently than cyanocobalamin to increase the level of one of the coenzyme forms of vitamin B12. The quantity of cobalamin detected following a small oral dose of mecobalamin is similar to the amount following administration of cyanocobalamin; but significantly more cobalamin accumulates in liver tissue following administration of methylcobalamin. Human urinary excretion of methylcobalamin is about one-third that of a similar dose of cyanocobalamin, indicating substantially greater tissue retention.

Indications

For the treatment of numbness, pain and paralysis due to peripheral neuropathies such as diabetic neuropathy and polyneuritis. For the treatment of Megaloblastic Anemia due to Vitamin B12 deficiency.

Dosage and administration

Adult: To be taken orally. 1 tablet, 3 times daily.

Contraindication

Methylcobalamin is contraindicated in those hypersensitive to any component of vitamin B12 containing product.

Side effects

Gastrointestinal: anorexia, nausea, vomiting or diarrhoea may occur infrequently

Skin reaction: Skin rashes or urticaria occurs very rarely.

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Manufactured and Distributed by Product Registration Holder:

Nova Laboratories Sdn. Bhd. (179832-D)

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Mecobamin

Symptoms and Treatment of Overdose

Gastrointestinal: anorexia, nausea, vomiting or diarrhoea may occur infrequently

Skin reaction: Skin rashes or urticaria occurs very rarely

If such symptoms occur, discontinue administration immediately and consult a doctor.

Drug Interactions

Antibiotics: The use of antibiotics may alter the intestinal microflora and may decrease the possible contribution of B12 by certain inhabitants of the microflora (e.g., *Lactobacillus* species) to the body's requirement for the vitamin. This may particularly be a problem for vegetarians. Garlic, onions, leeks, bananas, asparagus and artichokes, among other vegetables and fruits, contain inulins which promote the growth of certain colonic bacteria, including *Lactobacillus* species.

Cholestyramine: Cholestyramine may decrease the enterohepatic reabsorption of B12.

Colchicine: Colchicine may cause decreased absorption of B12.

Colestipol: Colestipol may decrease the enterohepatic reabsorption of B12.

H2 blockers (cimetidine, famotidine, nizatidine, ranitidine): Chronic use of H2 blockers may result in decreased absorption of vitamin B12 naturally found in food sources. They are unlikely to affect the absorption of supplemental B12.

Metformin: Metformin may decrease the absorption of vitamin B12. This possible effect may be reversed with oral calcium supplementation.

Nitrous oxide: Inhalation of the anesthetic agent nitrous oxide can produce a functional vitamin B12 deficiency. Nitrous oxide forms a complex with cobalt in methylcobalamin, the cofactor for methionine synthase, resulting in inactivation of the enzyme.

Para-aminosalicylic acid: Chronic use of the anti-tuberculosis drug may decrease the absorption of B12.

Potassium chloride: It has been reported that potassium chloride may decrease the absorption of dietary B12 in some.

Proton pump inhibitors (lansoprazole, omeprazole, pantoprazole, rabeprazole): Chronic use of proton pump inhibitors may result in decreased absorption of vitamin B12 naturally found in food sources. They are unlikely to affect the absorption of supplemental B12.

Birth control pills: Some studies have found reduced serum levels in oral contraceptive users, but others have found no effect despite use of oral contraceptives for up to 6 months. When oral contraceptive use is stopped, normalization of vitamin B12 levels usually occurs. Lower vitamin B12 serum levels seen with oral contraceptives probably are not clinically significant.

Storage Conditions

Store below 30°C.

Protect from light and moisture.

Warnings and Precautions

Keep out of reach of children. (*Jauhkan daripada capaian kanak-kanak*).

Treatment with mecobalamin should be discontinued if there is no response after one month of therapy.

Prolonged use of larger doses of Mecobamin tablet is not recommended for patients whose occupation requires handling mercury or its compound.

Pregnancy and Lactation

No abnormalities or signs of teratogenicity were noted in foetuses or neonates and newborns from animal studies performed on rats (before pregnancy, in the early stage of gestation, during period of organogenesis and during perinatal and lactating periods).

Shelf Life

3 years

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